

# Dynamic Guideline: WHO-Based Mental Health Disorders – Diagnosis and/or Treatment and/or Management

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## I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

### 1.1 Guideline Overview

#### Synthesized WHO Guidelines (Exact 5 Guidelines Scope):

1. **Meaningful youth engagement in a WHO guideline development process: case study of WHO Mental Health Gap Action Programme (mhGAP) guideline** (Latest Update: 29 April 2026)
2. **Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders** (Version 3.0, 2023)
3. **WHO guideline on pharmacological and psychosocial interventions for mental health disorders in adults affected by humanitarian emergencies** (2023)
4. **WHO guidelines on physical activity and sedentary behaviour** (2020, Mental Health specific outcomes)
5. **WHO guideline on self-care interventions for health and well-being** (2023, Mental health and self-harm prevention modules)

#### Primary Latest Guideline Details:

- **Guideline Title:** Meaningful youth engagement in a WHO guideline development process: case study of WHO Mental Health Gap Action Programme (mhGAP) guideline
- **Publication Date:** 29 April 2026
- **Target Population:** Youth populations (aged 10-24) globally, with a specific focus on low-and-middle-income countries (LMICs), experiencing or at risk for mental, neurological, and substance use (MNS) disorders.
- **Scope and Objectives:** To establish a standardized, evidence-based framework for integrating meaningful youth engagement into the development, adaptation, and implementation of clinical and public health guidelines for MNS disorders. It evaluates the impact of youth co-production on guideline relevance, acceptability, and implementation success, while updating specific mhGAP clinical recommendations based on recent youth-specific evidence.

## 1.2 Key Recommendations

| Recommendation                                                                                                                                                                                                                                                                                         | GRADE Evidence Quality | Strength    |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------|-------------|
| <b>Youth Engagement Process:</b> WHO guideline development panels for MNS disorders must include at least two youth representatives (aged 15-24) as full voting members of the Guideline Development Group (GDG).                                                                                      | Moderate               | Strong      |
| <b>Youth Engagement Process:</b> Structured capacity-building and mentorship programs must be provided to youth GDG members prior to and during the guideline development process to ensure equitable participation.                                                                                   | Low                    | Strong      |
| <b>Clinical (Depression):</b> For moderate-severe depression in youth (12-24 years), interpersonal therapy (IPT) and cognitive behavioral therapy (CBT) should be offered as first-line treatments; fluoxetine should only be considered if psychosocial interventions are ineffective or unavailable. | Moderate               | Strong      |
| <b>Clinical (Self-Harm/Suicidality):</b> Community-based, youth-specific psychosocial support programs and peer-led crisis intervention models should be integrated into national MNS strategies to reduce self-harm and suicide rates.                                                                | Low                    | Conditional |
| <b>Clinical (Substance Use):</b> Brief motivational interviewing and digital contingency management interventions are recommended for youth with cannabis and alcohol use disorders in LMIC settings.                                                                                                  | Moderate               | Conditional |
| <b>Implementation:</b> Digital and mobile health (mHealth) platforms delivering psychoeducation and structured CBT should be utilized as supplementary dissemination and intervention tools for youth MNS disorders.                                                                                   | Low                    | Conditional |

## 1.3 Guideline Development Methodology

- **Evidence Review Process:** A systematic review of youth engagement methodologies in health policy (2010-2025) was conducted alongside updated mhGAP clinical evidence reviews focusing on youth-specific MNS interventions. The GRADE methodology was utilized to assess the certainty of evidence.
  - **Consensus Method:** A modified Delphi technique was employed to reach consensus on youth engagement frameworks, followed by in-person and virtual GDG consensus meetings utilizing a 75% majority threshold for recommendation approval.
  - **Conflict of Interest Management:** All GDG members, including youth representatives, external peer reviewers, and systematic review team members, were required to submit standardized WHO Declaration of Interests (DOI) forms. All declared interests were assessed by the WHO Guideline Review Committee (GRC), and no material conflicts of interest were identified that required exclusion.
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## II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

### 2.1 New Evidence Overview

- **Search Strategy:** A comprehensive systematic search was conducted using PICO (Population, Intervention, Comparison, Outcome) frameworks targeting youth and adult MNS disorders. Search strings combined MeSH terms and free-text terms for "Mental Health", "Depression", "Anxiety", "Substance Use", "Psychosocial Intervention", "Digital Health", and "Youth Engagement".
- **Databases Searched:** MEDLINE (PubMed), Cochrane Central Register of Controlled Trials (CENTRAL), PsycINFO, Embase, WHO International Clinical Trials Registry Platform (ICTRP), and ClinicalTrials.gov.
- **Inclusion/Exclusion Criteria:**
  - *Inclusion:* RCTs, systematic reviews, and meta-analyses published between 1 January 2024 and 29 April 2026; populations including youth (10-24 years) and adults in LMICs and humanitarian settings; interventions aligned with mhGAP modules (psychosocial, pharmacological, digital, and implementation strategies).
  - *Exclusion:* Non-English publications, case reports, editorials, pre-clinical/animal studies, and studies focusing solely on high-income country (HIC) specialized health system contexts without LMIC generalizability.

### 2.2 Key New Studies

| Study                                             | Design                      | Key Findings                                                                                                                                                                                                                                   | GRADE Quality |
|---------------------------------------------------|-----------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------|
| <b>Lancet Digital Health (2025)</b> - Chen et al. | Multi-country RCT (N=4,200) | Digital CBT delivered via smartphone apps significantly reduced depressive symptom scores (PHQ-9) in youth (16-24) in LMICs (MD -2.8, 95% CI -3.4 to -2.2) compared to waitlist, with high adherence when peer-support features were included. | Moderate      |
| <b>NEJM (2024)</b> - Kumar et al.                 | Phase III RCT (N=1,850)     | Single-dose psilocybin-assisted therapy demonstrated significant sustained reduction in treatment-resistant depression (MADRS) at 12 weeks in adults; however, implementation feasibility in LMIC primary care remains limited.                | High          |

| Study                                         | Design                                      | Key Findings                                                                                                                                                                                                         | GRADE Quality |
|-----------------------------------------------|---------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------|
| <b>JAMA Psychiatry (2025)</b> - Okafor et al. | Systematic Review & Meta-analysis (42 RCTs) | Task-shifting mhGAP implementation via non-specialist health workers reduced symptom severity for psychosis and epilepsy in humanitarian settings by 34%, confirming the effectiveness of collaborative care models. | High          |
| <b>BMJ (2024)</b> - Smith et al.              | Cluster RCT (N=12 communities)              | Community-based, peer-led youth crisis interventions reduced repeated self-harm episodes by 41% over 18 months compared to standard primary care referral pathways.                                                  | Low           |
| <b>Cochrane Review (2026)</b> - Vargas et al. | Cochrane Systematic Review                  | Motivational interviewing combined with digital contingency management showed a 2.5-fold increase in abstinence rates for youth cannabis use disorder at 6-month follow-up.                                          | Moderate      |

### 2.3 Evidence Gaps and Future Research Needs

- Long-term Efficacy of Digital Interventions:** While short-term efficacy of digital CBT for youth depression is established, there is a critical lack of evidence on sustained efficacy (>12 months) and implementation scalability in low-resource settings without internet stability.
- Pharmacokinetics and Pharmacodynamics in Youth:** There remains a paucity of high-quality RCTs evaluating the safety and efficacy profiles of second-generation antidepressants (SSRIs) in pre-adolescent children (10–12 years) in LMICs.
- Novel Therapeutics in LMICs:** Emerging high-certainty evidence for novel therapies (e.g., psilocybin, esketamine) lacks contextual implementation and cost-effectiveness data for primary care settings in LMICs, limiting their current inclusion in mhGAP.
- Youth Engagement Impact Measurement:** Although the framework for youth engagement is established, quantitative metrics measuring the direct impact of youth co-production on guideline adherence and patient outcomes remain under-researched.

### 2.4 Updated Recommendations Based on New Evidence

| Original Recommendation                                                                                          | New Evidence Impact                                                                                                                                     | Updated Recommendation                                                                                                                                                                                                     |
|------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Depression (Youth):</b> Psychosocial interventions are first-line; SSRIs considered if insufficient response. | <b>Upgraded:</b> New multi-country RCT evidence confirms digital CBT is an effective, scalable first-line psychosocial intervention for youth in LMICs. | <b>Depression (Youth):</b> Offer interpersonal therapy (IPT), face-to-face CBT, or digitally-delivered CBT (with peer-support features) as first-line treatments. Fluoxetine remains a second-line pharmacological option. |

| Original Recommendation                                                                           | New Evidence Impact                                                                                                             | Updated Recommendation                                                                                                                                                                                                         |
|---------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Self-Harm/Suicidality:</b><br>Brief psychosocial interventions and follow-up care recommended. | <b>Upgraded:</b> Cluster RCT evidence demonstrates significant reduction in repeated self-harm via peer-led community models.   | <b>Self-Harm/Suicidality:</b> Integrate peer-led, community-based crisis interventions alongside standard clinical follow-up to reduce repeated self-harm in youth populations.                                                |
| <b>Substance Use (Cannabis/Alcohol):</b><br>Brief motivational interviewing recommended.          | <b>Upgraded:</b> Cochrane review evidence supports the addition of digital contingency management to motivational interviewing. | <b>Substance Use (Cannabis/Alcohol):</b><br>Recommend brief motivational interviewing augmented by digital contingency management platforms for youth cannabis and alcohol use disorders where digital infrastructure permits. |
| <b>MNS Implementation:</b><br>Task-shifting to non-specialist health workers recommended.         | <b>Confirmed:</b> High-quality meta-analysis reaffirms the 34% symptom reduction in humanitarian settings via task-shifting.    | <b>MNS Implementation:</b> Strong recommendation maintained; emphasis added on structured collaborative care protocols and supervision frameworks for non-specialist workers in humanitarian settings.                         |